

1. Administrative & Study Identification

Full Study Title: An Open-label, Multicenter, Dose-escalation, Randomized, Phase I Study to Evaluate Safety, Pharmacokinetics, Pharmacodynamics and Anti-Tumor Activity of R07567132, as a Single Agent and in Combination With Atezolizumab in Participants With Advanced and/or Metastatic Solid Tumors **Short Title/Acronym:** A Dose-escalation Study of R07567132 as Single Agent and in Combination With Atezolizumab in Participants With Advanced Solid Tumors / R07567132 Dose-escalation Study **Protocol Number:** BP44956 (2024-512839-70-00) **NCT Number:** NCT06537310 **Posting ID:** 1003612854593555875 **Sponsor Name:** Hoffmann-La Roche (Roche) **Investigational Product:** R07567132 (FAP-LTBR bi-specific stromal immunomodulatory agonist) and Atezolizumab (Trade Name: Tecentriq) **Phase of Development:** Phase I **Trial Status:** Active, Not Recruiting **Medical Monitor:** Clinical Operations Lead, Hoffmann-La Roche

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety, pharmacokinetics (PK), pharmacodynamics (PD) and preliminary clinical activity of R07567132 as a single agent and in combination with atezolizumab. **Secondary Objectives:** To determine the maximum tolerated dose (MTD) or optimal biological dose, characterize the pharmacokinetic/pharmacodynamic profile, and evaluate early signs of anti-tumor efficacy.

2.2 Background & Rationale While standard treatments like chemotherapy and immunotherapy have improved outcomes for cancer patients, they do not work for everyone or eventually stop working. New treatments are needed for advanced and metastatic solid tumors. R07567132 is a novel first-in-class bispecific antibody designed to bind bivalently to the lymphotoxin beta receptor (LTBR) and monovalently to the fibroblast activation protein (FAP). By restricting LTBR activation specifically to the tumor microenvironment via FAP targeting, R07567132 modulates the tumor stroma to increase immune cell infiltration and induce the formation of tertiary lymphoid structures (TLS), minimizing widespread toxicity. This study tests R07567132 both alone and in combination with the anti-PD-L1 therapy atezolizumab to explore whether it can enable and enhance the efficacy of cancer immunotherapies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Dose-escalation (Single-arm per cohort, active combination comparison) **Blinding:** Open-label **Randomization:** Randomized (across dose levels/cohorts as per protocol)

3.2 Planned Enrollment & Duration Target N\$: 34 Number of Sites: Multicenter (Global) **Estimated Study Start:** 16-Sep-2024 **Estimated Primary Completion:** 19-Jun-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female participants aged ≥ 18 years with body weight ≥ 40 kg. 2. Participants with selected locally advanced and/or metastatic solid tumors for whom standard therapy does not exist, or has proven to be ineffective or intolerable. 3. Eastern Cooperative Oncology Group (ECOG) performance status of 0 to 1 and life expectancy of at least 12 weeks. 4. Availability and willingness to provide a mandatory archival tumor specimen or (if not available) a fresh baseline biopsy. 5. Adequate cardiovascular, hematological, liver, and renal function, and a negative serum pregnancy test for females of childbearing potential.

4.2 Exclusion Criteria 1. Known central nervous system (CNS) tumors or metastases and leptomeningeal metastases. 2. Active second malignancy within 2 years prior to screening. 3. History or current clinically significant cardiovascular/cerebrovascular disease, or known clinically significant liver disease. 4. Active or history of autoimmune disease, or serious, uncontrolled infection. 5. Unresolved acute toxicity $\geq$ Grade 1 from prior therapy.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: R07567132 administered as monotherapy or in combination with atezolizumab (Trade Name: Tecentriq, ATC Code: L01FF05, EMA URL: https://www.ema.europa.eu/en/documents/product-information/tecentriq-epar-product-information_en.pdf) at doses and schedules specified for the respective dose-escalation cohorts. **Route of Administration:** Intravenous infusion (standard for monoclonal antibodies). **Duration of Treatment:** Until disease progression, unacceptable toxicity, or patient withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for advanced solid tumors. **Prohibited:** Concurrent investigational therapies and active systemic immunosuppressive medications.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, Medical History, Tumor Biopsy retrieval, Baseline Labs/Imaging
Treatment / Baseline	Day 1	First Dose of R07567132 \$\\pm\$ Atezolizumab, PK/PD Sampling, Safety Check
Interim	Weekly / Cycle specific	Frequent safety checks (at least weekly initially), PK/PD monitoring, Efficacy Assessments (Scans)
End of Study	Variable	Final Safety Visit, Exit Interview, Survival Follow-up

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Percentage of Participants with Adverse Events (AEs) Graded According to Common Terminology Criteria for Adverse Events Version 5.0 (CTCAE v5.0) and Percentage of Participants With Dose-limiting Toxicities (DLTs). **Measurement Method:** Clinical assessment and grading of toxicities per NCI CTCAE v5.0 criteria.

7.2 Secondary & Exploratory Endpoints

- * Pharmacokinetics (PK):** Maximum Concentration (Cmax) Derived for R07567132, Time of Maximum Concentration (Tmax), and Minimum Concentration (Cmin).
- * Pharmacodynamics (PD):** Changes in tumor microenvironment biomarkers, immune cell infiltration, and TLS induction.
- * Anti-Tumor Activity:** Objective Response Rate (ORR), Progression-Free Survival (PFS), and Duration of Response (DoR) based on RECIST v1.1.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring throughout the study, with patients evaluated at least every week during the initial phase to track unwanted side effects. **Serious Adverse Events (SAEs):** Standard 24-hour reporting timeline for all serious, life-threatening, or fatal events. **Data Monitoring Committee (DMC):** Internal and/or external Dose Escalation Committee to review safety data before clearing subsequent dose cohorts.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving at least one dose) and DLT-evaluable Set. **Sample Size Justification:** Driven by standard Phase 1 dose-escalation methodology (e.g., 3+3 design or Bayesian continual reassessment method) to identify the MTD. **Handling of Missing Data:** Standard handling for oncology

trials; efficacy generally evaluated for patients with at least one post-baseline scan.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring to ensure protocol adherence and data integrity. **Audit Trail:** Complete tracking of Electronic Case Report Forms (eCRF) and centralized data validation.

11. Study Identifiers & Governance

Primary ID: BP44956 **Registry Number:** NCT06537310 **Posting ID:** 1003612854593555875 **Sponsor/Lead Organization:** Hoffmann-La Roche (Roche) **Study Title:** A Dose-escalation Study of R07567132 as Single Agent and in Combination With Atezolizumab in Participants With Advanced Solid Tumors **Official Regulatory Title:** An Open-label, Multicenter, Dose-escalation, Randomized, Phase I Study to Evaluate Safety, Pharmacokinetics, Pharmacodynamics and Anti-Tumor Activity of R07567132, as a Single Agent and in Combination With Atezolizumab in Participants With Advanced and/or Metastatic Solid Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: Advanced and/or Metastatic Solid Tumors

Disease Areas: Cancer-Neoplasms **Preferred UMLS Name:**

Antineoplastic Agents -- therapeutic use -- clinical trials

Terminology Codes: MeSH: D009369 | HPO: HP:0002664 | SNOMED:

1240414004 **Diagnosis Threshold:** Histologically confirmed solid tumors refractory to standard care. **Medical Methodology:**

Immunomodulation of the tumor stroma / Checkpoint inhibition

Optimization Target: Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D)

13. Study Procedures & Methodology

Management Pathway: Experimental Dose-Escalation Oncology Pathway

Education Protocol (HCP): Site initiation visits and ongoing investigator training on DLT recognition and immune-related adverse events. **Education Protocol (Patient):** Informed consent process detailing the risks of Phase I experimental

immunotherapy. **Quality Audit Mechanism:** Independent central review of imaging and routine sponsor audits.

14. Observation & Follow-Up Schedule

Total Duration: Estimated 5 years (until primary completion in June 2029) **Onsite Visit Cadence:** Minimum weekly during the early treatment phase, adjusting to standard cycle lengths (e.g., Q3W or Q4W) thereafter. **Remote Monitoring Frequency:** Continuous monitoring between clinical visits as needed. **Checklist Review Interval:** End of each treatment cycle.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]		Lead		
Statistician	[Name]	_____	[DD-MMM-YYYY]				